

A Racing Heart: An Illness Narrative of Hyperthyroidism and Relapse

This illness narrative is based on an interview with “Lucia,” a pseudonym for my friend who has experienced hyperthyroidism, also known as Graves’ disease, and later relapse. Lucia is a 21-year-old international student from China studying in New York. Her story moves through three distinct phases: a first encounter with illness she was too young to fully understand, a relapse that brought chaos and collapse, and an eventual return to stability shaped by community, self-knowledge, and ongoing care.

Too Young to Understand

Lucia first experienced hyperthyroidism in 2019, when she was fifteen years old. At the beginning, she did not immediately recognize that something serious was happening. She was proud to stay clear-headed and highly active at school with even four to five hours of sleep. While she was sensitive to heat and impetuous that she had a hard time focusing on schoolwork, these weren’t really uncommon for a teenager. Her misunderstanding of the illness extended to her family as well as she recalled her mother blaming her for not studying hard enough. Only later did they realize that her difficulty focusing was related to her illness. The diagnosis came after a physical examination where her heart rate was found to be around 150 beats per minute, and she was eventually diagnosed with hyperthyroidism. Looking back, she described her most obvious symptoms as a state of extreme excitement and restlessness. Her body seemed to be constantly activated, as if it could not slow down. This shows how hyperthyroidism can be misread because its symptoms often appear as emotional or behavioral problems. Irritability, restlessness, poor concentration, and anxiety can easily be mistaken for personality flaws or lack of effort, rather than recognized as signs of a physiological condition.

When she first heard the diagnosis, she was surprised but not especially frightened. Because she was still young, she did not fully understand the seriousness of the condition. In fact, part of her reaction was almost lighthearted: she was happy that she did not have to take physical education classes anymore. At this early stage, hyperthyroidism was not yet something she understood as a long-term health issue but more of a sudden medical label attached to symptoms she had not fully interpreted. After the diagnosis, she began regular medical treatment. She had blood tests every month and thyroid ultrasounds every six months. She took Tapazole, and over the course of about six months to a year, her thyroid indicators gradually became more stable. After the pandemic, she was even able to start running again. This first part of her story seemed to follow a clear and reassuring path, a restitution story where something went wrong, medicine intervened, and the body recovered.

The Relapse

The relapse in 2024 shattered that sense of resolution. In September of that year, Lucia began to experience frequent palpitations again and went for a checkup. This time, the diagnosis felt entirely different. When she realized that her hyperthyroidism had returned, her first reaction was frustration and emotional collapse. The distress did not come only from fear of the disease itself, but also from the overwhelming thought of going through treatment again. As an international student in the United States, Lucia had to navigate a medical and insurance system she found difficult to understand, while language barriers added further friction to every interaction. She found herself caught in a negative feedback loop: her physical symptoms made it hard to study, her grades dropped, and her worsening anxiety seemed to intensify her illness. Eventually, she took a gap semester and returned to China for treatment. Although Lucia's

situation as an international student differs from the migrant workers in Holmes's ethnography, her decision to seek treatment in her home country reflects a related pattern. Holmes notes that many migrant workers in the United States return to Mexico for health care despite the hardships of travel, citing "economic, cultural, and linguistic reasons" for doing so (Holmes 2006, 1778). For Lucia, returning to China similarly meant entering a medical system that felt more familiar, affordable, and linguistically accessible. This decision brought some relief, but it also created new stress because it abruptly interrupted her academic plans.

Her treatment after relapse was similar to the first treatment in some ways. She still needed monthly checkups and medication adjustments. However, this time she took both Tapazole and levothyroxine. Although levothyroxine is usually used to treat hypothyroidism, taking both medications helped stabilize her thyroid indicators. Still, treatment was frustrating. The medication worked slowly, and she constantly worried about her lab results. Regular blood tests and changing indicators became a source of anxiety because they constantly reminded her that the illness still needed management. Kleinman argues that biomedicine often emphasizes "quantitative data" while giving little legitimacy to patients' values and meanings, so "meaning itself is not configured as a central focus or task of medicine" (1995, 32). Lucia's experience reflects this tension: her thyroid indicators could be measured and adjusted, but the numbers alone could not answer her worries about relapse, appearance, uncertainty, and long-term treatment choices. As her neck swelling became more obvious, she began to worry about her appearance and started exploring other treatment plans by visiting different doctors. She considered radioactive iodine treatment but eventually gave up the idea because of the possible risk of eye protrusion. She also considered surgery, but worried about scarring. One thing she

hoped for from doctors was for them to explain the advantages and disadvantages of different treatment options more clearly as all the doctors she visited gave different and sometimes contrasting opinions. Long-term medication, radioactive iodine treatment, and surgery all carry different consequences, with the first one having a high chance of relapse and the latter two developing into hypothyroidism. As a patient, she wanted to understand these options more fully instead of only being told what to do. Her experience shows that patients are not passive recipients of medical advice. They want to participate in decision-making, especially when the illness may relapse or require long-term care.

Learning To Live With It

Lucia has since recovered from the recurrence of hyperthyroidism: her biological indicators are back to normal and she no longer suffers from insomnia every night listening to her heart racing and pounding. But her understanding of hyperthyroidism has changed from seeing it as a temporary illness to seeing it as a condition that might require life-long management. She has become more responsible for her own health. She is now more sensitive to physical discomfort and more willing to go to the hospital when something felt wrong.

Hyperthyroidism also changed her daily habits. She ensures 8 hours of sleep everyday even if that means she needs to miss out on social events with friends or tell her supervisor at work she can't do overtime. She stops vaping and drinking alcohol, habits that she picked up during the summer internship before her relapse in order to get along with her colleagues. She avoids getting manicures or dyeing her hair because she worried that too much contact with chemical products might contribute to relapse. Whether or not this fear matches biomedical explanations

exactly, it shows how illness changes a person's everyday behavior. Small activities that once seemed ordinary can become connected to the possibility of getting sick again.

Her friends' experiences also became part of the frame through which Lucia understood her illness. Among Lucia's friends, several had similar experiences with thyroid-related illnesses: some had Graves' disease like her, while others had Hashimoto's-related hyperthyroidism. She started a group chat with these friends, where they shared symptoms, lab reports, treatment experiences, and own experience. For her, the most important help from these communities was emotional rather than informational. Seeing others go through similar experiences made her feel that she was not the only person dealing with this illness. The community gave her a sense of companionship and reassurance during a long and uncertain treatment process.

Lucia is now more at peace with her condition. The gap semester she took because of the illness gave her time to reflect and offered her new perspectives on what she wants to do in the future. In this sense, her story begins to move toward what Whitehead describes as a quest narrative, in which illness is not simply cured or resolved, but becomes something through which the patient creates meaning. Whitehead writes that quest narratives involve "accepting illness" and believing "that something is to be gained through the experience" (2006, 2237). For Lucia, this does not mean that relapse was positive or desirable. Rather, it means that she began to understand hyperthyroidism as part of her life instead of only as an interruption to it. What once seemed like a temporary condition became a long-term companion that required attention, negotiation, and a different kind of care than she had first imagined. Through this process, she learned to listen to her body, to seek out community, and to understand illness not as a single event that ends, but as an ongoing relationship with uncertainty and herself.

Bibliography

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